

TM-002: SESSION 1 FULL SCRIPT (MASTER REFERENCE)

TM-002 (T.M., 22) | TRAUMA-001 | CSP Pro Bono | THERAPIST ONLY — DO NOT DISTRIBUTE

TREATMENT ARC: 6-8 SESSION OVERVIEW

All 6 presenting themes (self-esteem, confidence, assertiveness, trust, anxiety/hypervigilance, letting go) share a single root: unresolved childhood trauma. The arc works from root outward to symptoms. EP result from Session 1 determines Physical vs Emotional delivery lane for the entire program.

SESSION	FOCUS	THERAPEUTIC GOAL	KEY TECHNIQUE	BUILD ON
S1	Assessment + First Induction	Screen, determine EP, build rapport, establish safe space + grounding anchor	Intake (L.O.V.E.), 36-Q suggestibility test, Theory of Mind, pre-induction speech, first induction (EP-branched), safe space visualization, grounding anchor (triple-fire)	Foundation
S2	The Root: Younger-Self Encounter	Access the core wound. Adult meets younger self in the Room. Identify the belief that was encoded.	Room Technique Phase 1-3 (build room, meet younger self, begin supportive dialogue). NOT regression. NEW safe room.	Safe space from S1
S3	Re-Parenting + Core Belief Reframe	Develop a fairer, more supportive response to earlier experiences	Room Technique Phase 4-5 (address unmet needs, build confidence). Core belief reframe.	Younger-self relationship from S2
S4	Integration + Anxiety Reduction	Carry the learning into adult behavior. Recalibrate threat detection.	Room Technique Phase 6 (integration). Anxiety dial/dimmer. Nervous system calming. Vagal toning.	Re-parenting from S3
S5	Trust + Assertiveness + Boundaries	Evaluate trust decisions. Install assertive voice. Boundary anchoring.	Circle of Trust visualization. Assertiveness chair. Boundary wall with gate (client controls).	Integrated perspective from S4
S6	Self-Hypnosis + Independence	Client runs the protocol alone. Reinforce all gains. Graduate.	Self-hypnosis teach (safe space + dial + affirmation). Relapse prevention. Future-pace.	All prior work
S7-S8	Contingency / Reinforcement	Only if needed. Address anything that surfaced. Reinforce.	Based on progress data.	As needed

Decision points:
After S1: EP informs Physical (direct language) vs Emotional (permissive inference) delivery style.
After S2: If younger-self encounter triggers strong catharsis, slow down. Add stabilization before S3.
After S3: If core belief has not shifted, repeat. Do not advance to integration prematurely.
After S4: If anxiety reduction is incomplete, reinforce before trust work in S5.
After S5: If client is self-sufficient, graduate at S6. If not, S7-S8 available.

CLINICAL RULES (entire treatment arc):
- Visit the past briefly to identify the encoded belief. Reframe it in present tense. Move forward. Do not loop in the past. The goal is behavior change in the present, not re-experiencing.
- Check expectations at every session (Melton): "What are you hoping will happen today?"
- Younger-self work is an optional component. Ask: "Does this approach feel useful to you?" Declining does not change access to treatment.
- Do not interpret images or feelings in trance as factual memories. Uncertain material should be approached without confirmation or dismissal. Memories may be symbolic, not literal.
- If client meets criteria for PTSD, consider referral to CPT/PE/EMDR specialist. Hypnotherapy complements but should not displace evidence-based trauma treatment.
- Never dismiss psychotherapy gains. Build on them. Client's prior work is a genuine asset, not competition.
- All self-report measures are SUBJECTIVE (HMI instructor rule). Use 1-10 scales. Do not promise biometric outcomes.

SESSION 1: FULL SCRIPT BEGINS

[Client: T.M., 22, childhood trauma, 6 presenting themes. CSP pro bono via HMI. Prior psychotherapy (reports significant progress). Requested male therapist for comfort discussing men's issues. EP: TBD (both branches fully scripted). This session is ASSESSMENT + FIRST INDUCTION ONLY. No deep trauma work. No regression. Build the foundation.]

STEP 1: WARM-UP (2-3 MIN)

T.M., welcome. Good to see you. Before we get into anything clinical, how was your day? What have you been up to?

[2-3 minutes of genuine human conversation. Ask about his week, his interests, anything non-clinical. This is not small talk — it is the first building block of trust. He requested a male therapist for comfort. Match his energy. Be direct but warm. Do not perform "therapist mode" yet. Let the rapport build naturally. Observe his baseline: posture, eye contact, energy level, tone of voice.]

Good. Thank you for making the time tonight. I know it takes a certain kind of courage to try something new, especially when you have already done therapeutic work before. That actually says something good about you — you are not someone who settles. You keep working on yourself until it feels right.

Session Frame (set expectations for tonight)

Here is how tonight works. This is our intake session. Think of it as the blueprint session. We are going to do five things.

First, we talk. I want to hear what brought you here, in your own words. No rush, no agenda. You lead.

Second, I ask some screening questions. Standard for every client, but especially important given what you have shared in your intake form. These are direct questions and I need honest answers so we can work safely together.

Third, I explain how hypnosis works and how the mind processes change. This gives you the full picture of what we are doing and why, so nothing feels mysterious or out of your control.

Fourth, I run a suggestibility assessment. It is not a test you pass or fail. It tells me how your mind processes information so I can tailor everything we do to how your brain works best.

Fifth, if you are comfortable, we do a first relaxation experience. This is optional. If at any point you want to stop, you raise a finger and we stop. No questions asked.

Sound good?

STEP 2: L.O.V.E. INTAKE — PRESENTING PROBLEM (10-15 MIN)

[L.O.V.E. = Listen, Observe, Verify, Empathize (Kappasinian). Listen: open-ended questions. Observe: body language, vocal shifts. Verify: reflect back and confirm accuracy. Empathize: feel WITH the client, show genuine emotional resonance.]

L — Listen (open-ended questions)

Tell me what brought you here. In your own words. Take your time.

[LISTEN. Do not interrupt. Let him tell the full story. Note the exact words he uses for each theme. His language reveals his subconscious framing. Watch for: body language shifts, vocal tone changes, eye contact patterns, emotional loading on specific words. Note which theme he leads with — that is usually what matters most right now. Write down his exact phrases. You will use those words back to him throughout the arc.]

[If he goes quiet or seems unsure how much to share: "You can share as much or as little as you are comfortable with tonight. There is no pressure to cover everything in the first session. We have time."]

O — Observe + V — Verify

Thank you for sharing that. That takes courage, especially with someone new.

[OBSERVE body language during his story. Note where he tensed, looked away, changed tone. VERIFY by reflecting back. Verification is confirming accuracy, not agreement. Do not minimize ("that's not so bad"). Do not say "I understand" unless you genuinely do. Better: "That makes sense" or "Thank you for being honest about that." Do not rush to fix or reframe yet. Sit with what he shared. Let the silence be comfortable.]

V — Verify (closed-ended anchors)

I want to make sure I heard you correctly. You have been working on this in psychotherapy, and it helped significantly. But there are some pieces that are still active. The self-esteem, the trust, the hypervigilance, the feeling of not being comfortable in your own skin. Those are still running in the background despite the insight you have gained. Is that accurate?

[REFLECT back. Let him correct you. His corrections tell you what matters most. If he corrects a word, use HIS word from now on. His corrections are the most important data you will get tonight.]

E — Empathize (connect, then explore)

That sounds exhausting. Carrying that level of alertness every day, even when part of you knows you are safe now. That takes a toll.

[EMPATHIZE. Show you FEEL the weight of what he described. Not "I understand" — emotional resonance. One sentence. Then explore.]

Can you give me a specific recent example? A situation in the last week or two where you noticed the anxiety or the hypervigilance showing up?

[EXPLORE. Get a concrete behavioral example. This grounds the work in real life, not abstract concepts. Also reveals which theme is most active RIGHT NOW. His answer may reshape the treatment arc priorities.]

And when that was happening, where did you feel it in your body? Was there a physical sensation? Chest tightness, stomach knot, jaw clenching, shoulder tension, anything physical?

[Body awareness question. If he can identify a physical location, that becomes a therapeutic target and a progress marker. If he cannot locate it physically, that is also data — it suggests he is more cognitively oriented than somatically oriented. Adjust induction approach accordingly. Physical EP clients typically locate sensation easily. Emotional EP clients may describe it more abstractly.]

Checking Expectations (Melton)

[MELTON: Always check expectations. Unchecked expectations are the number one cause of therapeutic failure. If the client expects one thing and experiences another, compliance drops even when the work is good. Ask directly. Do not assume.]

Before we go further, I want to ask you something important. What are you expecting from hypnotherapy? What do you think will be different about this compared to the psychotherapy you have done?

[His answer tells you what matters most. Listen for what he wants to be different. If he has unrealistic expectations (one-session fix, memory recovery, personality overhaul, mind control), address them honestly NOW. Common misconceptions: "You'll make me forget the trauma" (no). "You'll tell me why I'm like this" (no, your subconscious already knows). "I'll be a different person after" (no, you'll be more fully yourself).]

What do you already understand about these reactions, and what would you like to be different in how you feel or respond?

And what are you hoping hypnotherapy will add to the work you have already done?

[His answer to this last question is the most important thing he says tonight. It becomes the treatment frame. Write it down verbatim. It will anchor every therapeutic suggestion you deliver across the arc.]

That is a great answer. Let me tell you how I think about it, and you tell me if it resonates.

The therapy you have done already made real progress. You said so yourself. That work gave you understanding, coping tools, and the ability to talk about what happened. Those are genuine gains. Nothing we do here diminishes that work. We are building on it.

What you are describing now, the reactions that are still there despite that understanding, those often involve patterns that were encoded very early, before you had words for them. They live in the experiential part of the brain, not the analytical part. That is why talking about them helps but does not always resolve them completely.

What we do here is work with a deeply relaxed state where your mind is more open to experiencing things differently. Not just understanding a pattern, but feeling it shift. Think of it as adding an experiential layer to the insight you already have. We are not replacing what therapy gave you. We are completing it.

Does that make sense? Does that match what you are looking for?

[If yes: aligned. If no: explore further. Do not proceed until there is genuine alignment on what this process is and is not. Misalignment here undermines everything downstream.]

STEP 3: CO-CREATE THE PLAN WITH CLIENT

[MELTON: Preparation of the Critical Mind. Answer these 4 questions WITH the client before presenting the plan. This builds the cognitive pillars that open the critical mind for hypnotic suggestions.]

Before I share the plan, I want to make sure we are clear on four things.

First: what is the problem? Not the label, but what is actually happening in your daily life that you want to change?

[Let him answer. His answer is the behavioral target. Write it down verbatim.]

Second: why do you have it? Based on what you shared and what we discussed, why do you think these patterns are still running?

[He should connect it to early programming. If he does, the critical mind is opening. If he says "I don't know," bridge: "Based on what you told me, these patterns were encoded early, before you had a choice. They were survival strategies. Does that fit?"]

Third: what needs to change? Not everything at once. If you could shift one thing that would make the biggest difference right now, what would it be?

[His answer becomes the priority. Write it down.]

Fourth: how are we going to change it? This is where hypnosis comes in. We work with the 88 percent directly. We update the programming in the deeply relaxed state where the critical mind steps aside. Does that mechanism make sense to you?

[If yes: all 4 pillars built. Critical mind prepared. If any pillar is weak, strengthen before proceeding.]

Good. Now here is the specific plan.

Based on what you have shared, here is what I am hearing. You have six things that are interconnected. The self-esteem, the confidence, the assertiveness, the trust, the anxiety, and the difficulty letting go. And underneath all of them, there are experiences from childhood that encoded certain patterns into your subconscious.

Here is what I propose, and I want your input. We work together for six to eight sessions. In the first session, tonight, we do the assessment and give you a first experience of hypnosis. In the next sessions, we go deeper into the core patterns and begin shifting them. By session four or five, we start building your independence so you can use these tools on your own. And by session six, if things are tracking well, we graduate you.

We are not going to try to do everything at once. We go one layer at a time, checking in as we go. The pace is determined by how you respond, not by a calendar.

What do you think? Does that pace feel right? Is there anything you would want to prioritize or adjust?

[Let him respond. If he has a priority ("I really need the anxiety to come down first"), adjust the arc accordingly. His input gives him ownership of the process. Co-creation = better compliance = better outcomes. For trauma clients, agency is therapeutic in itself.]

Good. We are aligned. Let me run through the screening questions and then we will get into the education and assessment.

STEP 4: CRITICAL SCREENING (5-10 MIN)

[DO NOT SKIP ANY OF THESE. Childhood trauma requires thorough screening before any trance work. Ask directly. Do not soften the questions. Record all answers. If any red flag emerges, adjust or stop accordingly.]

Before we go any further, I need to ask you some screening questions. These are standard for every client, but especially important given what you have shared. I need honest answers so I can make sure we work safely together. None of these questions are meant to judge you. They are clinical data that helps me keep you safe.

Question 1: Active Professional Care

Are you currently seeing a therapist, psychologist, or psychiatrist? Are you in active treatment right now?

[If YES: get name, type of care, how often. Coordinate, do not conflict. If psychiatrist: check medications immediately. If NO: note that psychotherapy was past, not current. Ask when it ended and why. Ask: "What made you stop? Was it your decision or did treatment conclude?"]

Question 2: Medications

Are you currently taking any medications? Anything for anxiety, depression, sleep, or anything else?

[If anti-psychotic medication: STOP. Refer to licensed MHP. Do not proceed with hypnosis. If anti-anxiety or anti-depressant (SSRIs, SNRIs, benzodiazepines): proceed with awareness. Note medication name and dosage. These affect depth and emotional processing in trance. If recreational substances: note frequency. Do not judge. Note: cannabis and alcohol can affect suggestibility and trance depth.]

Question 3: Safety

I need to ask this directly, and I appreciate your honesty. Have you ever had thoughts of harming yourself or ending your life?

[If ACTIVE suicidal ideation: STOP the session. Provide 988 Suicide and Crisis Lifeline (call or text 988). Do not proceed with hypnosis. Refer to licensed MHP. Document the disclosure. If PAST ideation, now resolved: note it, proceed with caution. Ask: "When was the last time? What helped you through it?" If passive ideation ("Sometimes I wish I didn't exist but I wouldn't act on it"): note it, monitor throughout treatment, include safety check at every session close.]

Question 4: Trauma Scope

You mentioned experiences from when you were young. You do not need to share details today. But can you help me understand the general scope? Was this a single event, or something that happened over a period of time?

[Pause. Let him answer at his own pace. Do not fill the silence. If he needs time, give it to him.]

And without needing to go into specifics, did it involve physical harm, emotional harm, or both?

[Scope the trauma WITHOUT forcing disclosure. Session 1 is assessment, not processing. You need to know: single vs repeated, physical vs emotional vs both, who was involved (family, outside, authority figure). This determines the intensity and pacing of future sessions. Do NOT push for details he is not ready to share. If he says "I'd rather not get into that yet," respect it fully: "That is completely fine. We will get into it only when and if you are ready."]

Question 5: Triggers and Sensitivities

Is there anything I should be careful about during our work together? Words, topics, types of imagery, or situations that tend to trigger strong reactions for you?

[ESSENTIAL for trance work. If he says "closed spaces" — do not use confined visualizations. If "authority figures" — adjust your tone and positioning, soften directive language. If "physical touch imagery" — avoid body-contact language in progressive relaxation. If "darkness" — use light-filled safe space imagery. Write these down. Refer to them before every future session. These are non-negotiable boundaries.]

Question 6: Panic and Dissociation

Do you experience panic attacks? Episodes where your heart races, you cannot breathe, and it feels like something terrible is about to happen? And when you get very stressed or upset, do you ever feel like you are watching yourself from the outside, or like things are not quite real? Like there is a pane of glass between you and the world?

[Panic: if yes, adjust induction. Use open-space imagery, not enclosed. Include grounding anchors throughout. Give him the exit signal early. Dissociation: if yes, this is critical data. Dissociative clients require extra grounding in trance. Use body-awareness cues ("feel your feet, feel the chair") frequently. Monitor for blank staring or unresponsiveness during trance. If observed, ground immediately: "Feel your feet. Feel the chair beneath you. You are here. You are safe. Can you nod for me?"]

Confidentiality and Supervision Disclosure

Thank you. Those are the hard questions and you handled them well. Everything you share stays confidential, with two exceptions. If there is immediate danger to you or someone else, or if there is abuse of a child or elder, I have a legal obligation to report. Otherwise, this stays between us.

One thing to be aware of, because this is through the community service program: I may discuss our session with a clinical supervisor for educational and quality assurance purposes. Your identity is always protected. I would only use your initials. Is that okay with you?

[Get verbal acknowledgment of supervision disclosure. Note it. CSP compliance requires this disclosure before any clinical work begins.]

STEP 5: PRE-INDUCTION SPEECH — DISPELLING MYTHS (3-5 MIN)

[The 14-point pre-induction speech. This is foundational. It removes fear, corrects misconceptions, and builds informed consent. Do not skip any point. Deliver conversationally, not as a lecture. Pause between points for acknowledgment.]

Before we go any further, let me tell you exactly what hypnosis is and what it is not, so you know what to expect. No surprises. No mystery.

First: hypnosis is a natural state. You go in and out of it throughout your day without even realizing it. When you are absorbed in a movie and you forget you are sitting in a room. When you are driving and you arrive somewhere and think, "How did I get here?" When you are reading and the world falls away. Those are all natural trance states. What we are doing tonight is using that same natural ability intentionally.

Second: when I use the words Deep Sleep, that is a trigger for a hypnotic state. It is not actual sleep. You will be in a deeply relaxed state with your eyes closed, but you are not sleeping. You will be aware the entire time. Possibly more aware than you are right now.

Third: you will not go unconscious. You will not lose control. You will be aware of everything. Some people are even hyper-aware during hypnosis. They notice more, not less.

Fourth: you would not do or say anything under hypnosis that you would not do or say in your wide-awake state. This is not a truth serum. Your values, your boundaries, your judgment — all of that stays fully intact.

Fifth: at some point during the process, you may wonder, "Am I actually hypnotized?" And that is when you will know that you are. Because in the normal waking state, you do not question whether you are awake. The very act of wondering is the evidence.

Sixth: hypnosis is a suggestible state. That means your mind becomes more receptive to suggestions about your conscious goals. The goals you already have. I am not putting anything in your mind that you do not want there. I am helping your subconscious get on board with what your conscious mind already wants.

Seventh: you will hear everything I say. Your mind may wander. That is completely normal. It does not mean it is not working. Your subconscious is always listening, even when your conscious mind drifts. It will know when to come back.

Eighth: people experience hypnosis differently. Some feel light and tingly. Some feel heavy, like they are sinking into the chair. Some just feel relaxed and calm. There is no right or wrong way to experience it. Whatever you experience is exactly right for you.

Ninth: this is a cooperative process. It is a "do with" process, not a "do to" process. I am not doing something to you. We are doing this together. Your participation is what makes it work. The more you allow yourself to follow the process, the more effective it is.

Tenth: today we start the process of positive change. Not finish it. Start it. This first session lays the foundation. The real depth comes in the sessions that follow.

Eleventh: you can open your eyes at any time. You can speak. You can move. Nothing is locked or frozen. If you want to pause, raise your right index finger. I will respond immediately.

Twelfth: hypnosis does not make you weak or vulnerable. It is actually a state of focused power. Your mind is more concentrated, more receptive, more capable of change than in the normal waking state.

Thirteenth: everything we do tonight is reversible. If something does not feel right, we adjust. If a suggestion does not resonate, your subconscious simply does not accept it. You have a built-in filter that protects you.

Fourteenth: I work with the 88 percent of your mind that operates below the surface. You work with the 12 percent that is logic, reasoning, and willpower. Together, we have 100 percent. That is how change actually happens.

Any questions about any of that?

[PILLAR CHECK: 4 pillars should be built: (1) Expectations aligned, (2) 4 Critical Mind Qs answered, (3) Pre-induction myths removed, (4) Theory of Mind explained WHY hypnosis works for his issue. If solid, proceed. If weak, revisit.]

[Pause. Let him ask. Answer honestly. Common questions: "Will I remember everything?" (Yes.) "What if I can't be hypnotized?" (Everyone enters natural trance states. We just use that ability intentionally.) "What if something bad comes up?" (You have the exit signal. I will guide you to safety. We do not force anything.)]

STEP 6: THEORY OF MIND (5-7 MIN)

The Circle Model (conscious / critical mind / subconscious)

[Draw the circle on paper or whiteboard as you explain. Visual learners and kinesthetic processors both benefit from seeing the model. Point to each layer as you describe it. This is not optional — the visual anchor dramatically increases comprehension and buy-in.]

Let me show you how your mind is organized. I am going to draw this out so you can see it.

Picture a circle. At the very bottom is the primitive area. This is the oldest part of your brain. It has been there since before humans could speak. Its job is survival: fight, flight, or freeze. It makes decisions in milliseconds, faster than your conscious mind can think. It is the part that flinches before you know why. And for someone who experienced difficult things as a child, this part got very well-trained very early. It learned to stay alert, to watch for danger, to keep the guard up. That was intelligent. That kept you safe.

Above the primitive area is the subconscious mind. This is roughly 88 percent of your total processing power. Everything you have ever experienced is stored here. Your habits, your emotional patterns, your automatic reactions, your core beliefs about yourself and the world. Most of this programming was installed between the ages of zero and eight, when the mind was wide open. No filter. Whatever came in, got recorded. Positive associations, negative associations, all of it. That early programming is still running right now.

Between the subconscious and the outside world is a filter called the critical mind. Think of it as a gatekeeper. Its job is to compare every new idea against what is already stored in the subconscious. If the new idea matches the existing programming, it lets it through. If the new idea contradicts the programming, the critical mind blocks it. That is why willpower alone usually does not work for deep change. Your conscious mind says, "I want to trust people." But the critical mind checks the programming and finds, "Trust equals danger." So it blocks the change. Not because you are weak. Because the system is doing its job.

The top layer is the conscious mind. This is about 12 percent. Logic, reasoning, analysis, decision-making, willpower. The one that does not always work. Your conscious mind can analyze a problem perfectly. It can understand exactly why you react the way you do. But understanding alone does not change the programming. That is what you experienced in therapy — genuine understanding, genuine insight, but the underlying reactions still firing.

[Kappas: focus on behavior, not the story. The goal here is to explain the mechanism of change, not analyze the content of his trauma. Keep the explanation focused on how the system works, not what his specific programming contains.]

How Change Works (and why hypnosis matters)

Here is how a normal thought moves through the system. You have a thought in your conscious mind: "I want to feel safe around people." That thought travels down and hits the critical mind. The critical mind checks the database: "Is safety around people consistent with what is already stored?" If the stored programming says, "People are dangerous," the critical mind blocks the thought. Your conscious intention never reaches the subconscious where the actual change would need to happen.

What hypnosis does is temporarily relax that critical mind filter. Not destroy it. Not override it. Relax it. Just enough that new, healthier associations can reach the subconscious directly. We update the programming at the source. Instead of fighting against the 88 percent with the 12 percent, we get them aligned. Working together.

I work with the 88 percent. You work with the 12 percent. Together, we have 100 percent. That is how real change happens.

The Nervous System Frame (neuroscience grounding)

[Nasir: use neuroscience framing for clients who respond to evidence-based explanation. T.M. has psychotherapy background — he likely understands fight/flight/freeze. Build on existing knowledge rather than starting from scratch.]

Here is another way to understand what is happening. Your nervous system has a threat detection center — the amygdala. When you were a child and things were not safe, that system went on high alert. It learned to scan for danger constantly. That was intelligent adaptation. Your brain did exactly what it was supposed to do to protect you.

The challenge is that it never fully recalibrated. You are 22 now. Your circumstances have changed. But the detection system is still running at the sensitivity level it was set to years ago. That is not a character flaw. That is not weakness. That is a protective program that did its job and has not received the update that the situation has changed.

The vagus nerve connects your brain to your body. When the amygdala fires, the vagus nerve tells your body: speed up the heart, tighten the muscles, flood with cortisol, get ready. That is the hypervigilance you described. The body is responding to a signal that was accurate once but is not accurate now.

Part of what we are going to do is help that system learn that it can adjust the sensitivity. Not turn off. You still need appropriate awareness. But right now the dial is set very high. We want to bring it to a level that matches your actual circumstances today.

Does that resonate with your experience?

[Let him respond. His response tells you how well the frame landed. If he says "that's exactly it," you have buy-in. If he pushes back, explore what does not fit. His buy-in on the model determines how well he will accept therapeutic suggestions later.]

STEP 7: SUGGESTIBILITY TEST (5-10 MIN)

36-Question Assessment

Now I want to understand how your mind processes information. This is not a test you pass or fail. There is no good score or bad score. It is a diagnostic tool that tells me how to communicate with your subconscious most effectively.

There are two main processing styles. Physical and Emotional. Physical processors respond best to direct, literal, specific language. When I say, "Your hand is rising," it rises. Emotional processors respond best to inference and suggestion. When I say, "You may notice a sense of lightness," the lightness appears on its own. Everyone is a mix of both. I need to know your mix so I can tailor everything to how your brain is wired.

I am going to read you thirty-six statements. For each one, answer A or B based on which resonates more with how you actually are, not how you think you should be. There is no right or wrong answer. Go with your first instinct. Do not overthink it.

[Administer the 36-question suggestibility test. Score it. Determine Physical/Emotional percentage. Record the exact split. This informs delivery style for the entire treatment arc. Note: trauma clients may score differently under stress vs baseline. Score what you get today and be prepared to adjust if responses in trance tell a different story.]

Alright, let me score this.

[Score and announce result. Record percentage in session notes.]

IF PHYSICAL (50%+ Physical):

"You scored [X]% Physical, [Y]% Emotional. That means your subconscious responds most effectively to direct, specific language. When I give you a suggestion, I will be literal and concrete. 'Your hand IS rising. Your body IS relaxing.' No ambiguity. Your mind takes direct instructions and runs with them. That is a strength. It means we can be very precise in our work together."

IF EMOTIONAL (50%+ Emotional):

"You scored [X]% Emotional, [Y]% Physical. That means your subconscious works through inference and feeling. Instead of me telling you what to experience, I offer possibilities and your mind fills in what is right for you. 'You may notice a sense of warmth... you might feel a shift...' and your subconscious takes it from there. That is a strength. It means your mind is naturally creative and will customize everything to what is right for you."

IF BALANCED (within 10% of 50/50):

"You scored roughly balanced between Physical and Emotional. That gives us flexibility. Some suggestions will be direct, some will be more open-ended. Your mind responds to both. We will find the right mix as we work together."

Finger-Spread Suggestibility Test (scripted)

[This is both a suggestibility demonstration AND the setup for the PHS verify at the end of session. It shows the client that their subconscious can respond to suggestion. Do this BEFORE the induction so the arm-raising induction is not their first experience of ideomotor response.]

Before we move on, I want to show you something. Raise your right hand in front of your face, about ten inches away, fingers pointing up. Spread your fingers wide apart. As wide as they will go. Good.

Now stare at your middle finger. Focus all of your attention on that one finger. Take a deep breath in. And as you exhale slowly, you may begin to notice that your fingers are starting to draw together. Slowly. Gradually. As if there is a gentle magnetic pull bringing them closer.

You do not need to make them move. Just watch. Allow your subconscious to respond. The conscious mind just watches. The fingers move on their own.

Notice that. Your subconscious responded to a suggestion without any effort from your conscious mind. That is exactly how hypnosis works. Your subconscious is already demonstrating that it is ready to participate in this process.

[If fingers moved: strong suggestibility response. Acknowledge it. If fingers did not move noticeably: "Even subtle movement counts. The subconscious is engaged. It may show itself more strongly as we continue." Do not make the client feel they failed.]

Good. Lower your hand. Any questions about that?

STEP 8: PHYSIOLOGICAL CHANGES (TRANSITIONAL)

[This bridges the conscious education phase and the induction. It primes the client to expect and accept the physical signs of trance onset. Delivered confidently and matter-of-factly. This is not a suggestion — it is a factual statement about what will happen next.]

Now as you begin to enter the hypnotic state, you are going to notice a series of changes taking place in your body. These are natural physiological responses. They happen to everyone.

You may notice your breathing slows and deepens on its own. Your pulse may slow slightly. Your muscles may start to feel heavy or warm. Your mouth and lips may feel dry, and you may notice an urge to swallow. All normal. All signs that your body is shifting from its alert state into a more relaxed state.

I will notice these changes before you do. When you notice them, that is your confirmation that the process is already working.

Pay particular attention to your eyelids. You may notice a very slight fluttering sensation — your eyes moving underneath your eyelids. When that happens, we are entering the first stage of hypnosis.

[This sets up ratification. When the client notices the physiological changes, they believe they are in hypnosis — which deepens the trance. Self-fulfilling in the best sense.]

STEP 9: CONSENT + SAFETY SETUP (BEFORE INDUCTION)

TRAUMA-SPECIFIC RULES FOR ENTIRE TRANCE SEQUENCE:

- NO age regression. No "go back to a time when..."
- NO forced imagery. If he resists a visualization, follow his lead.
- Permission language throughout: "with your consent," "only as far as comfortable"
- Safe space must be NEW — NOT a childhood place. Client designs it.
- Exit signal given BEFORE starting. "Raise your right index finger to pause."
- If abreaction occurs: DEEPEN trance (do NOT count out), guide to safe space, use grounding language.
- PHS wording: always include "safely and comfortably."
- Monitor for dissociation: blank staring, sudden stillness, unresponsiveness. If observed, ground immediately with body-awareness cues.
- Gentle count out — never rapid.

Are you comfortable proceeding with the relaxation exercise? There is absolutely no pressure. We can save this for next session if you prefer.

[Get explicit verbal consent. If he hesitates, do not push. "We can absolutely wait. There is no timeline here." Consent is non-negotiable, especially with trauma clients. Forcing a trance experience recreates the power dynamic that caused the original wound.]

Good. Before we begin, I want to give you a tool. If at any point during the relaxation you want to pause, or if something comes up that feels too intense, raise your right index finger. That is your signal. The moment I see it, I will guide you to a comfortable place and we will slow down. You are in control throughout. This is your process.

Find a comfortable position. Feet flat on the floor. Hands resting on your thighs. Take a deep breath in through your nose. Hold it for a moment. And exhale slowly through your mouth. One more. Deep breath in. And exhale. Good.

STEP 10: ARM-RAISING INDUCTION (EP-BRANCHED)

DELIVER THE BRANCH THAT MATCHES THE EP RESULT.

Physical EP: Use the Physical Arm-Raising script immediately below.

Emotional EP: Skip to the Emotional Arm-Raising script further down.

Both are fully scripted. Use the one that matches. The other is reference for future sessions if needed.

PHYSICAL ARM-RAISING INDUCTION (literal, direct, power words)

[PHYSICAL EP ONLY. Direct commands. Literal language. Power words. "Your hand IS rising." No "you may notice." Physical suggestibles need to FEEL the hypnosis in their body. The physical experience IS the evidence that convinces them.]

Sit up straight. Feet flat on the floor. Hands resting on your thighs.

Place your right arm on the armrest, or on your thigh if there is no armrest. Let it rest there naturally. Feel its weight pressing down. Notice the contact between your arm and the surface beneath it.

Now as your elbow presses down into the surface, the muscles in your upper arm are beginning to tighten. Feel them engage. A gentle contraction. Building. As that contraction increases, from your elbow down to your fingertips, your lower arm and hand are beginning to lift and rise.

Higher and higher. Lighter and lighter. As light as a feather. Lifting and rising. Pulling and moving. Moving up off the surface. With every breath you take in, your arm and hand lift and rise higher.

Lifting. Rising. Higher. Lighter. Pulling. Moving up. With every breath now, higher and higher.

Feel the lightness spreading from your hand through your forearm. As if the bones are hollow, as if the muscles are weightless. Lifting. Rising. Your hand is moving now. Upward. Toward your face.

And now your hand starts to twist and turn inward at the wrist. Turning. Rotating. Almost as if there is a magnetic pull between your hand and your forehead. A pull that gets stronger with every breath. Stronger and stronger.

Lifting, rising, twisting, turning upward and inward. The magnetic pull is drawing your hand toward your face. Closer with every breath. You can feel it. The pull is getting stronger.

With every breath you take in now, your arm and hand lift and rise. The pull between your hand and your forehead is undeniable. Your hand is moving. Closer. Closer.

Feel your feet against the floor. Feel your back against the chair. Those anchors are solid. And your hand continues to rise. Drawn upward. Drawn inward. Toward your face.

At the same time, you may notice your right hand feels lighter than your left. The left hand is heavy, pressing down. The right hand is light, rising up. Feel that difference. The heavier the left becomes, the lighter the right becomes.

Now when your hand comes all the way up to touch your face, the moment you make skin contact, you will nod your head, and you will reach your peak of suggestibility and enter the hypnotic sleep.

Rising. Closer. Almost there. The magnetic pull is almost complete.

[Wait for contact. Do not rush. If the hand stalls, add breathing cues: "Take a deep breath in. And as you exhale, the hand rises higher." If the hand does not move after 2 minutes, gently assist: "Allow me to guide your hand." Physical EP clients respond to physical cues.]

Good. Hand touching face. Nod your head. Deep Sleep.

Each and every time I suggest the words Deep Sleep to you, for the purpose of hypnosis and with your permission, you will enter this state very quickly, calmly, safely and comfortably, and deeply, and the physical body will relax.

[First PHS installed. Note the time. Trauma adaptation: "safely and comfortably" included.]

EMOTIONAL ARM-RAISING INDUCTION (inferential, permissive, misdirection)

[EMOTIONAL EP ONLY. Inferential language. Permissive suggestions. Misdirection technique. "You may notice..." "I wonder if..." Emotional suggestibles respond to possibility, not command. Their subconscious fills in the details.]

Find a comfortable position. Let your hands rest naturally wherever they want to be. Close your eyes if that feels comfortable, or let your gaze soften on a spot in front of you.

I want you to use your imagination now. Imagine that your left hand is becoming lighter and lighter. Very, very light. As light as a feather. And then even lighter than that. Imagine a gentle breeze is catching it from underneath, lifting it. So light that gravity almost does not apply.

And at the same time, you may notice that your right hand feels heavier than your left. Heavier. More grounded. Pressing down into whatever surface it is resting on. The heavier the right hand becomes, the lighter the left hand becomes. Feel that difference growing.

You may notice something interesting happening. Your left hand may begin to drift. Not because you are making it move. Because lightness rises. It is simply physics. Allow whatever happens to happen. Do not resist it and do not force it. Just allow.

Feel your feet against the floor. Feel your back against the chair. Those are your anchors. And your left hand continues to float. Rising. Drifting. Lighter with every breath. As if helium balloons were tied to each finger, gently pulling upward.

And as that hand floats higher, you may notice it beginning to turn. Twisting gently at the wrist. Turning inward. Moving toward your face. As if something is drawing it there. A pull you cannot quite name but can definitely feel.

Lifting, rising, twisting, turning upward and inward. Toward your face. At its own pace. In its own way. Your subconscious is guiding the movement now. The conscious mind just watches. Curious. Observing.

I wonder if you will notice that the closer your hand gets to your face, the heavier your eyelids become. As if they are connected by an invisible thread. Hand rises, eyelids drop. Closer and closer. Heavier and heavier.

And I wonder, when your hand finally touches your face, whether it will feel more like the hand was drawn to the face, or the face was drawn to the hand. I wonder which one you would choose.

Now when your hand comes all the way up to touch your face, the moment you make skin contact, allow your eyes to close completely, nod your head, and let yourself sink into the most comfortable state of relaxation you have ever experienced.

[Wait for contact. Emotional EP clients may take longer. The hand movement may be subtle. Do not rush or add too many suggestions — let the subconscious work. If the hand moves even slightly, acknowledge it: "There. That movement. Your subconscious is already responding." If eyes are already closed, simply wait for hand contact.]

Good. Hand touching face. Eyes closed. Nod. Deep Sleep.

Each and every time I suggest the words Deep Sleep to you, for the purpose of hypnosis and with your permission, you will allow yourself to enter this state safely and comfortably, at your own pace, quickly, calmly, and deeply, and the body and mind will settle into a peaceful rest.

[First PHS installed. Note the time. Emotional PHS uses "allow yourself" and "at your own pace" — permissive language matching the EP]

STEP 11: 5-0 COUNT + PHS

Hand down to your lap now. Gently. Comfortably. Let it rest.

Five. We are beginning to go down now. Deeper and deeper into the state of hypnosis. Your body already knows the way. It has done this a thousand times before sleep. This is no different. Just deeper.

Four. Physically releasing, relaxing, and letting go. Every muscle fiber standing down. Shoulders dropping. Jaw softening. Arms heavy. The body settling into the chair as if it wants to melt into it.

Three. An emotional calmness now. The conscious mind stepping back. The subconscious coming forward. You are safe here. Nothing to analyze. Nothing to figure out. Just allowing.

Two. You are mentally relaxing now. Thoughts slowing down. The noise in the background of your mind fading. Quieter. Still. Deeper.

One. Going deeper and deeper. Past the surface. Past the everyday chatter. Into the quiet place underneath.

And zero is Deep Sleep. Deep Sleep.

Each and every time I suggest the words Deep Sleep to you, for the purpose of hypnosis and with your permission, you will enter this state very quickly, calmly, safely and comfortably, and deeply, and the physical body will relax.

[PHS repeated. Second installation. The repetition is intentional and critical — each PHS delivery strengthens the post-hypnotic anchor.]

STEP 12: HAND-TO-FOREHEAD CHALLENGE (PHYSICAL EP ONLY)

[PHYSICAL EP ONLY. Skip this section for Emotional EP clients. Replace with an additional progressive relaxation pass or reactional deepener cycle.]

Your hand is still resting against your forehead. That connection is solid. Feel how firmly the palm is pressed against the skin. Like two pieces of marble fused together. One solid piece. Your hand and your forehead are connected now.

The more you try to pull your hand away, the stronger it presses against your forehead. You may try. Go ahead. Try to pull it away. And the harder you try, the more firmly it sticks. That is your subconscious holding it there. Your conscious mind may try to move it, but the 88 percent is stronger than the 12 percent.

[Brief test, 3-5 seconds. If the hand does not move: excellent depth. If it moves slightly: "Good. Stop trying. Feel it lock back in place." If it comes away completely: move on without drawing attention to it. The test is a depth gauge, not a pass/fail]

Good. Stop trying. The hand relaxes. Five. Four. Three. Two. One. Zero. Deep Sleep. Deeper now. Twice as deep as before.

Hand down gently to your lap. Let it rest. Heavy. Warm. Comfortable.

Each and every time I suggest the words Deep Sleep, for the purpose of hypnosis and with your permission, safely and comfortably, quickly, calmly, and deeply, and the physical body relaxes.

[PHS repeated. Third installation.]

STEP 13: REACTIONAL DEEPENER (2 CYCLES)

[For trauma clients: 2 cycles only on the first session. Do not overload. The reactional deepener is fast and disorienting — limit the intensity on the first experience. Standard non-trauma clients get 3-4 cycles. Increase to 3 cycles in Session 2 once trust is established.]

In a moment I am going to ask you to open your eyes. Just briefly. You will find a spot on the ceiling or the wall and stare at it. When I say the words Deep Sleep, your eyes will close again and you will go twice as deep as you are right now. Ready?

Open your eyes.

[Wait 2-3 seconds. Observe: pupil dilation, eye flutter, slow blinking = good depth indicators.]

Deep Sleep. Twice as deep. Eyes closed. Sinking down. Deeper than before. Safe and comfortable.

One more time. Open your eyes.

[Wait 2-3 seconds.]

Deep Sleep. Twice as deep. Deeper than you have been. The body heavy. The mind quiet. Safe. Comfortable. Held.

Each and every time I suggest the words Deep Sleep, for the purpose of hypnosis and with your permission, safely and comfortably, quickly, calmly, and deeply, and the physical body relaxes completely.

STEP 14: OPTIONAL — HEAVY/LIGHT DEEPENER (PHYSICAL EP ONLY)

[PHYSICAL EP ONLY. Skip for Emotional EP or if session is running long. This deepener builds somnambulistic depth but is not required for Session 1. Use clinical judgment: if the client is already deep (slow breathing, limp body, delayed responses), skip this and proceed to progressive relaxation.]

In a moment I am going to ask you to extend both arms out at shoulder height. Keep your eyes closed.

Arms out now. Good. Turn one palm open, facing up toward the ceiling. The other hand, make a fist with the thumb pointing up toward the ceiling.

On the open palm, I am placing a heavy bag of sand. Feel it. Feel the weight pressing down into your palm. Getting heavier. Heavier. Every second it gets heavier. Pulling your hand down toward your lap. Pressing down. Heavy. So heavy.

On the thumb of the other hand, I am tying helium balloons. One. Two. Three. Four. Five balloons. Feel them pulling up. Lifting your hand. Lighter. Rising. Your hand wants to float. The balloons are pulling it up and the sand is pressing down and the difference between the two hands is getting larger and larger with every breath.

[Pause 10 seconds. Observe the differential movement. The greater the separation between hands, the deeper the trance. Even slight movement is a positive indicator.]

Good. Release. Let the balloons go. Let the sand disappear. Arms down to your lap. Deep Sleep. Deeper now. Twice as deep. Comfortable. Safe.

Each and every time, for the purpose of hypnosis, with your permission, safely and comfortably, quickly, calmly, and deeply, and the physical body relaxes.

[PHS repeated. Fifth installation (if used).]

STEP 15: PROGRESSIVE RELAXATION (FULL BODY SCAN, DELTA-PACED)

[Delta pacing: 2-3 second pauses between phrases. Slow. Rhythmic. Match your breathing to his. Drop your voice half a register. This is where the body fully surrenders. For trauma clients, include grounding language ("you are safe," "you are supported") woven throughout. Do not use language that implies constraint or being held down.]

Now as you relax in your chair, you feel the rhythm of your breath. [Pause.] You feel connected inside. [Pause.] You are giving yourself permission to allow me to guide you into an even deeper state of relaxation. [Pause.] And that permission is yours to give and yours to take back at any time.

Allowing a sense of relaxation to wash down over your scalp now. [Pause.] Like warm water slowly spreading across the top of your head. [Pause.] Down over your forehead. [Pause.] Your temples. [Pause.] Smoothing out. Releasing. [Pause.]

Your eyelids softening. [Pause.] The tiny muscles around your eyes letting go of every bit of tension they have been holding. [Pause.] Your cheeks relaxing. [Pause.] Your jaw unclenching. [Pause.] Lips slightly parted. [Pause.] The entire face letting go of whatever it has been carrying today, this week, maybe much longer than that. [Pause.]

Pay no attention to any outside sounds, except my voice. [Pause.] These outside sounds are everyday sounds of life, and they cannot distract or disturb you. [Pause.] If anything, they tend to relax you even more. [Pause.] Each outside sound reminds you that the world is continuing normally while you rest deeply here. [Pause.]

Your neck softens now. [Pause.] Feel the muscles on either side of your spine release their grip. [Pause.] Your shoulders drop to their natural resting position. [Pause.] Not held up. Not braced. Not guarding. Just resting where gravity places them. [Pause.]

Down through your upper arms. [Pause.] Your elbows. [Pause.] Your forearms. [Pause.] Your wrists. [Pause.] Your hands opening gently. [Pause.] Your fingers softly curled. [Pause.] All the way down to your fingertips. [Pause.] All the tension draining out through your fingertips like water running off the edge of a leaf. [Pause.]

Your chest opens. [Pause.] Breathing deepens on its own. [Pause.] You do not need to control it. [Pause.] Your body knows how to breathe. [Pause.] It has been doing it since the moment you were born. [Pause.] Let it breathe for you now. [Pause.]

Down into your solar plexus. [Pause.] That center of your body where emotions sometimes gather. [Pause.] Letting go there. [Pause.] Not forcing anything out. Just allowing it to soften. [Pause.]

Your belly relaxes. [Pause.] Your lower back releases. [Pause.] Your hips soften. [Pause.] Your pelvis settles. [Pause.] All the protective tension your body has been carrying, all day, all week, maybe all year, letting go now. [Pause.] With every exhale, a little more releases. [Pause.]

Down through your thighs. [Pause.] Heavy. [Pause.] Your knees. [Pause.] Your shins. [Pause.] Your calves. [Pause.] Your ankles. [Pause.] Your heels pressing gently into the floor. [Pause.] The arches of your feet. [Pause.] Your toes. [Pause.]

From the top of your head to the tips of your toes, every system in your body is shifting into rest now. [Pause.] The alarm system turning down. [Pause.] Not off. Just down. [Pause.] To a level that matches this moment. [Pause.] This moment is safe. [Pause.] You are supported. [Pause.] You are held by the chair beneath you and the ground beneath that. [Pause.]

Your breathing becomes very deep, gentle, and rhythmic now. [Pause.] And the drowsy, sleepy, daydreaming feeling of relaxation takes over completely. [Pause.] Deeper with every breath. [Pause.] More comfortable with every exhale. [Pause.]

Five. [Long pause.] Four. [Long pause.] Three. [Long pause.] Two. [Long pause.] One. [Long pause.] Zero. Deep Sleep.

Each and every time I suggest the words Deep Sleep to you, for the purpose of hypnosis and with your permission, you will enter this state very quickly, calmly, safely and comfortably, and deeply, and the physical body relaxes.

[PHS repeated. This is the deepest installation point. The body is fully relaxed and the subconscious is maximally receptive.]

STEP 16: STAIRCASE DEEPENER (20 STEPS)

[Full 20-step staircase. For trauma clients: the destination at the bottom is an open, safe, sunlit space — NOT a dark room, NOT an enclosed area, NOT a childhood location. Use natural, open imagery: garden, meadow, beach, hilltop. The staircase itself is well-lit, solid, and feels secure. Include safety language: "sturdy," "well-lit," "smooth handrails."]

Now in your mind's eye, you find yourself standing at the top of a beautiful staircase. [Pause.] It is well-lit. Warm light fills the space around you. [Pause.] The staircase has sturdy, smooth handrails on both sides. [Pause.] You can feel the polished wood under your hand. [Pause.] It has twenty steps leading down. [Pause.] And with each step you take, you will deepen your state of hypnosis. [Pause.] Safely. Comfortably. At your own pace. [Pause.]

Place your hand on the railing. [Pause.] Take the first step. Left foot at nineteen. [Pause.] Feel the solid step beneath your foot. Stable. Secure. [Pause.]

Right foot at eighteen. [Pause.] Already deeper. [Pause.]

Twice as deep at seventeen. [Pause.] The light around you is warm and golden. [Pause.]

Sixteen. [Pause.] Fifteen. [Pause.] Fourteen. [Pause.] Each step taking you deeper. More relaxed. More at ease. [Pause.]

Thirteen. [Pause.] Twice as deep at twelve. [Pause.] Halfway now. The staircase curves gently. The air feels softer. Warmer. [Pause.]

Eleven. [Pause.] Ten. [Pause.] Letting go at nine. [Pause.] Any remaining tension melting away with each step. [Pause.]

Eight. [Pause.] Twice as deep at seven. [Pause.] You can feel the depth now. Your body is so relaxed it barely feels like it belongs to you. [Pause.]

Six. [Pause.] Five. [Pause.] Deeper still at four. [Pause.]

Three. [Pause.] Almost there. [Pause.]

Two. [Pause.]

One. [Pause.]

And zero. Deep Sleep. [Pause.] The deepest you have been. [Pause.]

At the bottom of the staircase, you step out into a wide, open, sunlit space. [Pause.] Fresh air on your skin. [Pause.] The sky above you is clear and blue. [Pause.] The ground beneath your feet is soft and warm. [Pause.] There are no walls. No ceiling. Just open space and gentle warmth and complete safety. [Pause.]

This is deeper than you have ever been. And you are completely safe here.

Each and every time I suggest the words Deep Sleep, for the purpose of hypnosis, with your permission, safely and comfortably, quickly, calmly, and deeply, and the physical body relaxes.

[PHS repeated. Deepest installation point.]

STEP 17: THERAPEUTIC SUGGESTIONS — SAFE SPACE + GROUNDING ANCHOR

[Session 1 therapeutic content is LIMITED to: (1) building a safe space the client designs, (2) installing a physical grounding anchor (triple-fire), and (3) a post-hypnotic suggestion for between sessions. NO trauma processing. NO inner child work. NO belief reframing. Those come in S2-S4. Tonight is foundation only.]

Safe Space Construction (client designs) — Say It, See It, Feel It

[MELTON suggestion formula: SAY it (describe verbally). SEE it (client visualizes every detail). FEEL it (client embodies the emotional and physical experience). All 3 channels = strongest installation.]

[This safe space becomes THE ROOM for younger-self work in Sessions 2-3. The client MUST design it. NOT a childhood memory. A brand new space that belongs to the 22-year-old adult. Engage all five senses. Let him choose every detail. If Physical EP, prompt with specific sensory questions. If Emotional EP, offer possibilities and let him discover. Record every detail he describes — you will reference it verbatim in future sessions.]

Now I want you to create something. A space. A room that belongs entirely to you. [Pause.] No one else has ever been here. [Pause.] No one can enter without your invitation. [Pause.] This is a new space. Not a memory. Not a place from your past. [Pause.] Something your mind is building right now, from the ground up, just for you. [Pause.]

IF PHYSICAL EP:

"What does this room look like? Tell me. What color are the walls? What is the floor made of? Is there a window? What do you see through it? Is there furniture? What kind?" [Wait for responses. Use his answers to build the space in detail. Physical EP clients give concrete, specific answers.]

IF EMOTIONAL EP:

"Allow this space to take shape on its own. You may notice colors beginning to appear... textures forming... a sense of the space around you defining itself. Let it emerge at whatever pace feels right. There is no rush. Your subconscious knows exactly what this space needs to be." [Give time. Let him discover. Do not impose details.]

[SEE IT: engage visual channel — colors, textures, light, window view.]

Notice the temperature in this room. [Pause.] Is it warm? Cool? Just right? [Pause.] What does the air feel like against your skin? [Pause.] Are there sounds? [Pause.] Maybe silence. [Pause.] Maybe something gentle in the background. [Pause.] Music. Water. Wind. Whatever feels right for this space. [Pause.]

Is there a scent? [Pause.] Something clean, something natural, something comforting? [Pause.] Cedar. Rain. Fresh linen. Something that tells your brain: this place is mine. [Pause.]

Is there a place to sit? [Pause.] A chair, a couch, a cushion on the floor, a window seat? [Pause.] Find the spot that feels exactly right and settle into it. [Pause.] Feel how it supports your body perfectly. [Pause.]

[FEEL IT: engage emotional + physical channel — safety, ownership, calm, body sensations.]

Feel how this space holds you. [Pause.] You decide what enters and what stays outside. [Pause.] You control the door. [Pause.] You control the windows. [Pause.] You control every aspect of this environment. [Pause.] It is entirely yours. Nothing that makes you uncomfortable can exist here. Not because the walls keep it out, but because you decide what belongs. [Pause.]

This space is being installed in your subconscious mind right now. [Pause.] Permanently. [Pause.] It will be here whenever you need it. [Pause.] In session. Between sessions. In the middle of the night. In the middle of a difficult conversation. [Pause.] Close your eyes. Take one breath. And you are here. [Pause.]

Subconscious mind, when this room is fully established and you feel completely safe and comfortable inside it, allow a nod to happen.

[Wait for ideomotor nod. This is the first ideomotor check. If no nod comes after 30 seconds, gently: "Take your time. There is no rush. When it feels ready, let it happen." If still no nod after another 30 seconds, proceed. Some clients do not give strong ideomotor signals on the first session. That is okay. Do not make it a failure point.]

Good. That is your room now. Installed. Permanent. Available anytime you need it. No one can take it away.

Grounding Anchor (triple-fire installation)

Before we come back, I want to give you a physical tool. Something you can use anytime, anywhere, without anyone knowing. A portable version of this calm.

Press your thumb and index finger together on your left hand. [Pause.] Feel that pressure. Really feel the contact between those two fingers. [Pause.] That physical sensation is now being linked to everything you are feeling in this moment. This safety. This room. This calm. This deep relaxation. All of it connected to that simple press of thumb and finger. [Pause.]

[First fire. Hold for 5 seconds.]

Now release. [Pause.] And press them together again. [Pause.] Feel the calm deepen. Feel the room around you become more vivid. [Pause.] The colors brighter. The textures clearer. The safety stronger. [Pause.] The anchor is strengthening with each press. [Pause.]

[Second fire. Hold for 5 seconds.]

Release. [Pause.] One more time. Press. [Pause.] And this time, as you press, take a slow breath in. [Pause.] And as you exhale, feel every bit of tension in your body drain out through your feet and into the ground beneath you. [Pause.] The anchor. The breath. The calm. The room. All linked together now. Permanently. [Pause.]

[Third fire. Triple installation makes the anchor significantly stronger than a single fire. The combination of physical stimulus (finger press) + breath + visualization creates a multi-channel anchor.]

Subconscious mind, when this grounding anchor is fully installed and ready for use in the waking state, allow a nod to happen.

[Wait for nod.]

Good. Anytime the anxiety shows up. Anytime the hypervigilance kicks in. Anytime you feel the old programming trying to run the show. Press your thumb and finger, take one breath, and the calm returns. It may not be as deep as it is right now. But it will be there. A physical anchor you carry with you everywhere you go.

STEP 18: POST-HYPNOTIC SUGGESTION (BETWEEN SESSIONS)

One more thing before we come back. Between now and our next session, you are going to notice some things.

You may notice moments of unexpected calm. Moments where the volume on the anxiety is just a little bit lower than usual. Moments where you catch yourself feeling at ease and think, "That is different." When those moments happen, notice them. Do not analyze them. Do not ask why. Just notice. Each one is evidence that the work we did tonight is active.

You may notice that you sleep a little differently this week. Maybe deeper. Maybe with more vivid dreams. That is your subconscious processing what we did tonight. Allow it. The dreams are not problems. They are processing.

And I am giving your dreaming mind a specific instruction. You will remember to remember your dreams. And in your early morning dreams, your subconscious will vent out anything that no longer serves you. Old fears, old protective responses that have done their job and can now be released. These venting dreams may be vivid. They may be strange. They may be emotional. That is exactly what they are supposed to be. Each one clears space. Each one restores capacity. When you wake remembering a vivid dream, that is proof the process is working.

[Venting dream suggestion (standard HMI first session). Gives the subconscious a structured channel for processing. The dream becomes evidence of progress, building compliance and confidence.]

You may notice that in certain situations where you would normally tense up or go on high alert, there is a split second of space. A tiny gap between the trigger and the reaction. That gap is new. That gap is the beginning of change. It will grow wider with each session.

These suggestions are firmly embedded in your subconscious mind. They grow stronger by the day, by the hour, by the minute. The work continues even when you are not thinking about it. Your subconscious is processing around the clock.

STEP 19: COUNT OUT (GENTLE, GROUNDING, TRAUMA-ADAPTED)

[Gentle count out. NOT rapid 1-2-3-4-5. Trauma clients need grounding, not abrupt awakening. Re-establish body awareness before opening eyes. Include door-closing imagery to seal the session. Take your time. If he seems disoriented upon opening eyes, extend the grounding: "Take a moment. Feel your feet. Feel the chair. You are here in the room with me."]

It is now time to come back up out of the state of hypnosis, feeling very calm and relaxed. [Pause.] You will bring with you everything we installed tonight. [Pause.] The safe room. [Pause.] The grounding anchor. [Pause.] The post-hypnotic suggestions. [Pause.] All permanent. All yours. [Pause.] Each and every time I suggest the words Deep Sleep to you, for the purpose of hypnosis and with your permission, you will enter this state safely and comfortably, quickly, calmly, and deeply, and the physical body will relax. That suggestion is sealed now.

[Final PHS seal before count out.]

Zero. Let us shut that door to the subconscious mind now. [Pause.] Shutting it gently. Sealing in every positive suggestion. [Pause.] Blocking all negatives from entering. [Pause.] The door is closed. Everything good stays inside. Everything we built tonight is protected.

One. Slowly and calmly coming up out of the state of hypnosis now. [Pause.] Feel your body. [Pause.] Becoming aware of the weight of your body in the chair. [Pause.] The temperature of the room. [Pause.]

Two. Physically aware and mentally alert. [Pause.] Taking a deep breath in. [Pause.] Energy returning. [Pause.] Feeling refreshed. Not groggy. Clear. [Pause.]

Three. Coming up even higher now. [Pause.] Hearing the sounds around you. [Pause.] The hum of the room. [Pause.] Feeling more present with each breath. [Pause.]

Four. Almost back. [Pause.] Eyes beginning to open. [Pause.] Fingers and toes moving gently. [Pause.] Feel your feet on the floor. [Pause.] Solid. Grounded. [Pause.]

And five. Eyes open. Wide awake. [Pause.] Fully alert. Grounded. Present. Safe. Welcome back.

One, two, three, four, five. Eyes open and wide awake.

Take a moment. There is no rush. Let your eyes adjust to the light. Feel the room around you.

STEP 20: PHS VERIFY (FINGER-SPREAD RE-HYPNOSIS TEST)

Welcome back. How do you feel?

[Give him 30-60 seconds. Let him orient. First trance can be emotional, especially for trauma clients. If he tears up: "That is a completely normal response. Take your time." If he seems disoriented: "You are here in the room with me. Take a breath. Feel the chair." Do not rush this transition.]

Let us do a quick test to see how the suggestion installed. This will only take a moment.

Raise your right hand. Palm facing you. Spread your fingers wide. Focus on your middle finger. Take a deep breath.

Your fingers are beginning to draw together. [Pause.] Your hand is moving toward your face. [Pause.] Breathing deepening. [Pause.] Eyelids heavy. [Pause.] Closer. [Pause.] Closer. [Pause.] When your hand touches your face, nod.

Deep Sleep.

[Time the response. Compare to the original induction duration. Faster = stronger PHS installation. Record: Induction time = ___. PHS verify time = ___. The ratio tells you how well the anchor set.]

Good. The post-hypnotic suggestion is installed and working. That response will get faster with each session. The acceleration is one of the ways we measure progress.

Each and every time I suggest the words Deep Sleep, for the purpose of hypnosis, with your permission, safely and comfortably, quickly, calmly, and deeply, and the physical body relaxes.

[PHS repeated one final time during verify. Then count out gently.]

Zero. One. Two. Three. Four. Five. Eyes open and wide awake. Welcome back.

STEP 21: POST-TRANCE DEBRIEF (3 QUESTIONS)

Question 1: Overall Experience

How do you feel right now? Take your time.

[Let him answer fully. Give him space. Common responses: relaxed, calm, heavy, light, emotional, surprised, "that was weird," "I'm not sure anything happened." ALL are normal. Do not judge or correct any response. If he says "I don't think I was hypnotized," respond: "The fact that you wondered means you were. And the PHS test just confirmed it. Your subconscious responded exactly as it should."]

Question 2: Specific Observations

What did you notice during the relaxation? Were there any images, sensations, or feelings that stood out?

[Do NOT interpret his experience. Do not say "that means..." Just listen and acknowledge. If he describes his safe room, that is excellent data — note every detail for future sessions. If he describes unexpected emotions, validate: "That is your subconscious beginning to process. Completely normal." If he describes nothing special: "That is fine. The subconscious works below the level of conscious awareness. The changes happen whether you notice them or not."]

Question 3: Self-Reported Depth

How deep did you feel you went? If zero is wide awake and ten is the deepest possible relaxation, where would you put yourself?

[Record self-reported depth ___/10. This is subjective, not clinical measurement. But it is useful for comparison across sessions. Do not correct him. If he says 3/10, do not say "you were actually deeper." Accept his number. It will increase as trust builds.]

STEP 22: SMART GOAL BASELINES (6 MEASURES)

[SMART goals are SUBJECTIVE (HMI instructor rule). Use 1-10 scales only. Do not promise biometric/tracker-based outcomes. Biometrics = "additional reference data" at most. These baselines will be re-measured periodically to track progress. Record all numbers tonight.]

Before we wrap up, I want to get some baseline numbers. These are entirely subjective. There are no wrong answers. Just where you are right now, tonight, in this moment.

On a scale of 1 to 10, where 10 is the best it could be:

How comfortable do you feel being yourself right now? Just being in your own skin, without performing or protecting?

[Record: Self-esteem baseline ___/10]

How easy is it for you to set a boundary or say no when you need to? One is nearly impossible, ten is effortless.

[Record: Assertiveness baseline ___/10]

On a typical day, how much does anxiety affect your decisions and behavior? One is barely noticeable, ten is it runs everything.

[Record: Anxiety baseline ___/10 (NOTE: high number = more anxiety = worse. This scale is inverted from the others. Clarify this in notes.)]

How much do you trust the people closest to you right now? One is very little, ten is completely.

[Record: Trust baseline ___/10]

How often do you feel genuinely at ease, just existing without being on guard? One is almost never, ten is almost always.

[Record: Comfort/ease baseline ___/10]

And how much does the past still influence how you feel and react in the present? One is barely at all, ten is it shapes everything.

[Record: Past influence baseline ___/10 (NOTE: high number = more past control = worse. This scale is also inverted. Clarify in notes.)]

Thank you. Those numbers give us a clear starting point. We will check them again periodically so you can see the movement. The numbers do not need to change overnight. They change as the subconscious reprograms. That takes sessions, not minutes.

STEP 23: HOMEWORK + NEXT STEPS

Here is your homework between now and our next session. Two things. Just two. I want to keep this simple.

First: the grounding anchor. Thumb and index finger on your left hand, press together, one deep breath. Use it whenever the anxiety shows up or the hypervigilance kicks in. Use it before a difficult conversation. Use it when you cannot sleep. Use it in the middle of a meeting when you feel your guard going up. The more you use it, the stronger it gets. It is a muscle you are building. Every time you press and breathe, the connection between that physical gesture and the calm we installed tonight gets stronger.

Second: at some point this week, sit quietly for two or three minutes, close your eyes, and visit the room you built tonight. Walk around it. Notice the details. Maybe add something new. A book. A blanket. A plant. Whatever feels right. Strengthen that space. It is going to become an important tool in our work together. The more real it becomes to you, the more effective it is as a therapeutic resource.

That is it. Two things. No journals, no tracking sheets, no complicated protocols. Just the anchor and the room.

[Keep homework simple for Session 1. Trauma clients with therapy history have homework fatigue risk. Too much = overwhelm = dropout. Build complexity gradually. If he asks "should I journal?" say: "Only if that feels helpful to you right now. It is not required. If you do journal, write about what you noticed this week — not about the past, about the present. What moments of calm surprised you? What felt different?" If he asks about meditation apps or recordings: "Those are fine as supplements, but the two homework items I gave you are the priority."]

Closing

How are you feeling about everything we covered tonight?

[Let him respond fully. Listen for: relief ("I already feel different"), surprise ("that was not what I expected"), skepticism ("I'm not sure it worked"), emotional processing (quiet, reflective). All are normal and healthy responses. Match your response to his energy. If he is energized, match energy. If he is quiet and reflective, be quiet and reflective.]

You did really well tonight. The fact that your subconscious accepted the safe space and the grounding anchor on the first session tells me your mind is ready for this work. That is not always the case. Some people need two or three sessions before their subconscious starts cooperating. Yours was on board from the start. That is a strong foundation to build on.

For our next session, we will go deeper. We will use that safe room you built and begin the therapeutic work on the core issues you described tonight. But Session 1 was the foundation. And it is solid.

Let us schedule session two. I would suggest one to two weeks from now. What works for your schedule?

[Schedule Session 2. Provide Calendly link: calendly.com/jeeth-vrishihypno/90min. CSP clients: 60-minute standard, 90-minute if clinical need warrants. Document rationale if using 90-minute slot.]

Safety Net + Crisis Information

Last thing. This is important, so I want you to hear it clearly. If between now and our next session anything comes up that feels overwhelming — if a memory surfaces that you were not expecting, or if the emotions feel too intense — you have three options.

First, use the anchor. Thumb and finger, one breath. Go to the room. That is your first line.

Second, reach out to me. You have my email. I will respond within 24 hours.

Third, if it is ever urgent, if you are in crisis or if you are having thoughts of harming yourself, call or text 988. That is the Suicide and Crisis Lifeline. Available 24 hours a day, 7 days a week. You are not alone in this process. There is always someone available.

Thank you for trusting me with this, T.M. I will see you at session two.

STEP 24: POST-SESSION THERAPIST CHECKLIST

COMPLETE ALL OF THESE IMMEDIATELY AFTER CLIENT DISCONNECTS (while memory is fresh):

#	TASK	DONE
1	Complete SOAP notes (Subjective, Objective, Assessment, Plan). Include verbatim client quotes for key moments.	☐
2	Record EP result: Physical ___% / Emotional ___%. Note any discrepancy between test score and observed trance response.	☐
3	Record induction time: ___min ___sec. Record PHS verify time: ___min ___sec. Calculate ratio.	☐
4	Record self-reported depth: ___/10.	☐
5	Record all 6 SMART goal baselines: Self-esteem ___/10, Assertiveness ___/10, Anxiety ___/10, Trust ___/10, Comfort ___/10, Past influence ___/10.	☐
6	Note safe room description in detail (colors, textures, sounds, scents, furniture, window view). You will reference this verbatim in S2.	☐
7	Note all screening answers: medications (names, doses), active care (therapist name, frequency), safety (ideation history), trauma scope (single/repeated, physical/emotional/both, who), triggers, dissociation, panic.	☐
8	Note any triggers or sensitivities he identified. Add to session prep checklist for ALL future sessions.	☐
9	Note which presenting theme he led with. This is likely the priority for S2.	☐
10	Note his exact words for what he hopes hypnotherapy will add. This is the treatment frame.	☐
11	Note any ideomotor responses: nods during safe space? Nods during anchor? Finger movement during finger-spread test? Arm movement during induction?	☐
12	Update TM-002.json with all session data (EP, baselines, screening, safe room, timestamps).	☐
13	Generate client documents: <code>python scripts/generate_client_docs.py data/clients/TM-002.json</code>	☐
14	Log CCH hours to HMI (CSP compliance). Record: date, duration, session number, client initials only.	☐
15	If any red flags emerged (active ideation, anti-psychotic medication, severe dissociation, PTSD criteria): document and consult supervisor BEFORE scheduling S2.	☐
16	Reflection: What went well? What would you adjust for S2? What surprised you? What did you learn about this client that changes your approach?	☐
17	Prepare S2 script outline: review safe room details, select Room Technique phases 1-3, adapt delivery to EP result, include trigger-avoidance list.	☐